جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ولاء محمد فرج سباعى

المحافظة : القاهره **الحي / المدينة :** دار السلام **العنوان :** ..

رقم البطاقة : 27810230103945 **رقم التليفون :** 01126466255 **تاريخ تسجيل الإستمارة :** 21-10-2026

العمر(تاريخ الميلاد) : 23-10-1978 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ولاء محمد فرج صابر
National ID: 27810230103945

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد،،

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

تتشرف بأن توضح فيما يلي تقريرا عن حالته

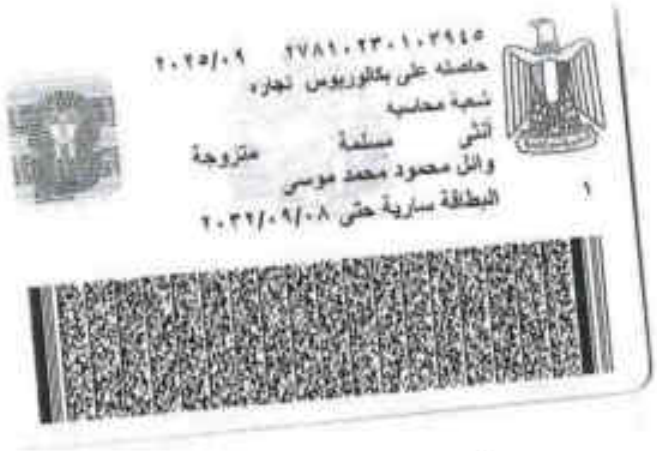
تاريخ وساعة الدخول : ---:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد أنة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : --:--
	الطبيب المعالج : د/شيماء لاشين


بطاقة الرقم الشخصي
ولاء
محمد فرج سباعى فرج
٣ ش ماهر عبدالرحيم من فرج يوسف
دار السلام - القاهره
٢٧٨ ١٠ ٢٣ ٠١ ٠٣٩ ٤٥
LJ2221939


٢٧٨١٠٢٣٠١٠٣٩٤٥ ٢٠٢٥/٠٩
حاصله على بكالوريوس تجاره
شعبة محاسبه
انثى مسلمة متزوجة
وائل محمود محمد موسى
البطاقة سارية حتى ٢٠٣٢/٠٩/٠٨
١

GUSTAVE ROUSSY
Gustave Roussy International Egypt - Pathology Lab
جوستاف روسي للتشخيص – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	ولاء محمد فرج	**Case No.**	344973
Sex/Age	Female 47 Y	**Patient No.**	25351
Registered Date	30/7/2026 09:36:10 AM	**Referred By Dr.**	Interventional Radiology
Validation Date	17/8/2026 02:36:28 PM	**Branch**	Gustave Roussy
Reporting Date	18/8/2026 09:42:11 AM	**Pathology No.**	2396/26

Pathology Report

Clinical Data: Left breast multicentric mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 30/07/2026, at 1:40 pm. Cold ischemia time is 20 seconds.

Gross Examination:
- Fragmented grey, white tissue cores, ranging in size between 0.2 cm & 1 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
- **Histologic type:** Invasive duct carcinoma with mucinous component.
- **Histologic grade:** 2 (differentiation 2, pleomorphism 2, mitosis 2).
- **Ductal carcinoma in situ:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.

Results:
*** IPT (All controls showed appropriate reactivity):**
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in <1% of tumor cells (score 2/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score

Name: ولاء محمد فرح سماعي	
MR. : 13340	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نتشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول : تاريخ وساعة الخروج :

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient is a 47-year-old female with a known history of metastatic breast cancer involving bone. She presented with limited mobility, prompting magnetic resonance imaging of the lumbosacral spine on 15 July 2026, which demonstrated multiple bony lesions concerning for metastatic disease and a partially imaged large sacral mass.

Core needle biopsy of the left breast revealed invasive ductal carcinoma, grade II, with an estrogen-receptor score of 8/8, progesterone-receptor score of 2/8, HER2 +1, and Ki-67 proliferation index of 40 %. Bilateral sonomammography performed on 28 July 2026 showed a BIRADS 3 lesion in the left breast and a BIRADS 5 lesion with ipsilateral pathological lymph nodes (levels I-III). PET-CT on 23 July 2026 demonstrated FDG-avid multicentric lesions in the left breast consistent with the primary tumor, as well as multiple FDG-avid nodal, bone, and liver metastases.

The multidisciplinary team recommended weekly taxol, palliative radiotherapy to the sacral bone mass, and zoledronic acid (Zometa) administered every three months.

تحريرا في ,

التاريخ : 23-09-2026 الوقت :

الطبيب المعالج :

يعتمد ,

Patient Name	ولاء محمد فرج	**Case No.**	344973
Sex/Age	Female 47 Y	**Patient No.**	25351
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Validation Date	17/8/2026 02:36:28 PM	**Branch**	Gustave Roussy
Reporting Date	18/8/2026 09:42:11 AM	**Pathology No.**	2396/26

1+).
- Ki67%: 40%.

Conclusion:

Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma with mucinous component, Grade 2.

IPT:
- ER: Positive (score 8/8).
- PR: Negative (score 2/8).
- Her2: Low (score 1+).
- Ki67%: 40%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

*** Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

مركز الجيزة للأشعة
GIZA SCAN

أ.د/ حســـــن الكيكى
أ.د/ محمد عبد اللطيف
د./ رانـــيـــا حـــمدى
د./ أيمـــن عبد الوهاب

ID : 11167143	NAME: ولاء محمد فرج سباعي
Date : 15/07/2026	Prof.Dr : Ashraf Ismail

MRI EXAMINATION OF THE LUMBO-SACRAL SPINE

Technique of examination:

- Sagittal T1 & T2 WIs
- Axial T2 WIs
- No IV contrast was given.

MRI Findings:

- There are multiple dorsal and lumbar vertebral body masses noted showing low T1 and low to intermediate T2 signal concerning for neoplastic lesions. No vertebral body collapse, no evident extra-vertebral component or vertebral canal extension. No neural compromise or foraminal narrowing.
- Also noted is a partially imaged left sacral mass showing the same signal characteristics. The outline of the mass can be seen in full on the scout images and approximately measures 5cm. Dedicated imaging is advised to evaluate for local infiltration and neural compromise if clinically found relevant. The scout images also show multiple concerning iliac bone small lesions.
- Preserved lumbar lordotic curvature.
- Normal diameters of the bony lumbar spinal canal.
- Preserved height and T2 signal brightness of the lumbar intervertebral discs.
- No evidence of disc herniations or significant bulges.
- Normal MRI appearance of the conus medullaris and cauda equina nerve roots.
- No paraspinal signal abnormalities.
- Uterine fibroids noted.

Opinion:

- ***Multiple bony lesions concerning for metastatic disease for further evaluation. No lumbar neural compromise.***
- ***Partially imaged large sacral mass – better evaluated by dedicated imaging.***

Kind regards,
Dr. Khaled Adel, Consultant radiologist

٥ ش مراد - ميدان الجيزة تليفاكس : ٣٧٧٤٦٠٢٠ - ٣٧٧٤٦٠٣١ - ٣٥٦٨٨٩٥٤ موبايل: ٠١١٤٠٧٧٢٢٦٨
E-mail:giza.scan@yahoo.com

Patient's Name:	**Walaa Mohammed Farag Sabai Farag**
Date:	**2026-07-28**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left palpable mass

Findings:

Right breast.

- Heterogenously dense breast parenchyma (ACR c)
- UOQ oval obscured margins lesions
- UOQ grouped macrocalcifications
- **Ultrasound** demonstrated a circumscribed oval hypoechoic lesion at 10 oclock zone B measuring 17 x 10 mm
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma (ACR c)
- large infiltritave dense irregular speculated mass with pathological pleomorphic calcifications involving most of the UOQ and upper central region
- thickened skin , coarsened trabeculations
- **Ultrasound** demonstrated central and outer breast 10- 5 o'clock large irregular speculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study measuring about 7.8 x 4.3 cm reaching the nipple.
- Thickened skin reaching 10 mm with coarsened trabecular and minimal subdermal edema.
- Multiple innumerable pathological axillary LNs are noted at level 1, 2 & 3, the largest at level 1 measuring 2.3 x 17 mm

Conclusion:-

- ***Right breast (BIRADS 3): 10 oclock probably benign lesion, likely fibroadenoma***
- ***Left breast (BIRADS 5): highly suspicious lesion with ipsilateral pathological level 1, 2 , 3 axillary nodes for histopathological correlation***

dr farah elshobaky

Pat. Name: Walaa Mohamed Farag Sebaey	Pat. ID: 3607480
Procedure Name: PET - CT. Nuclear	Date: 23/07/2026

Whole Body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- Female patient with vertebral lesions on MRI.
- **PET/CT requested for assessment.**

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.



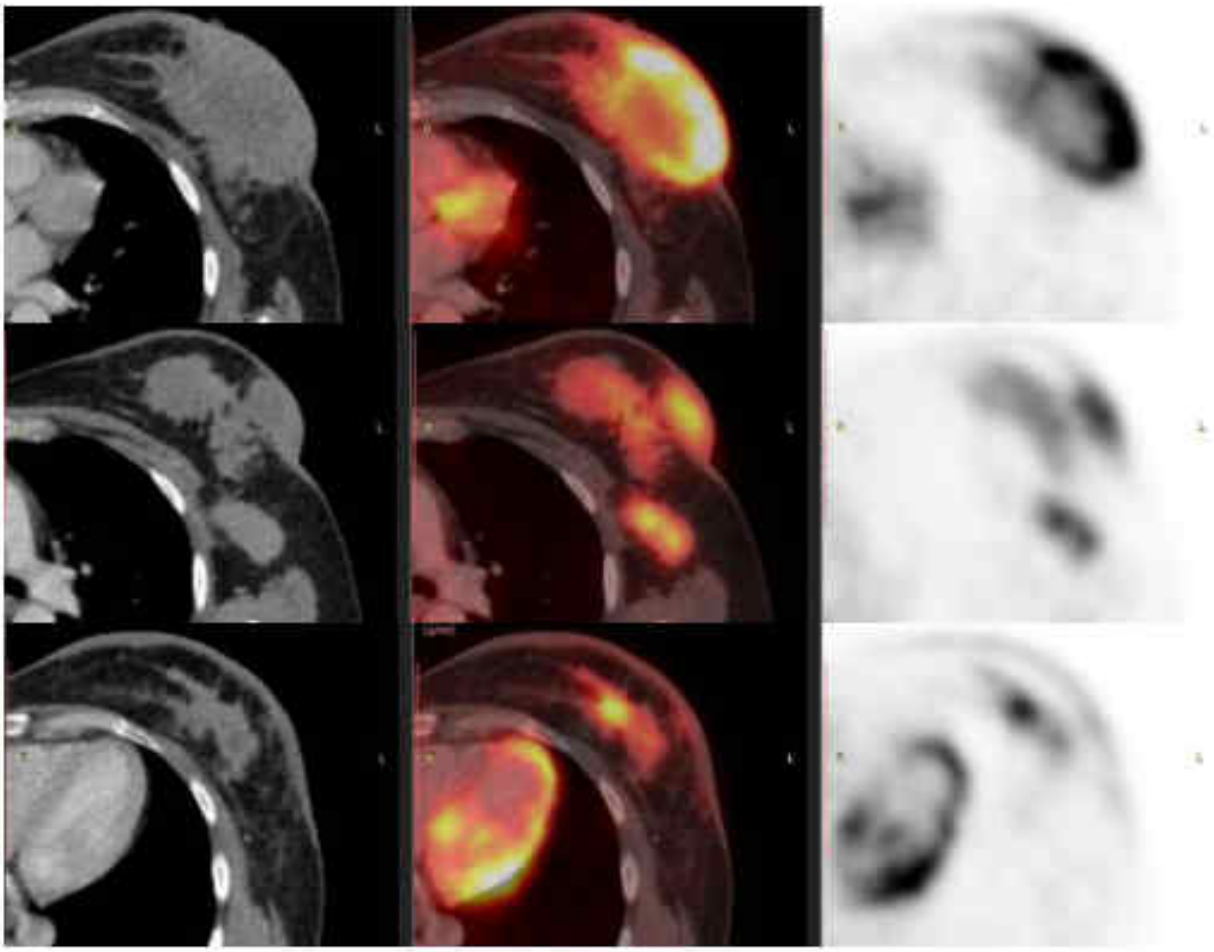
Report

Hepatic reference activity: 2.5

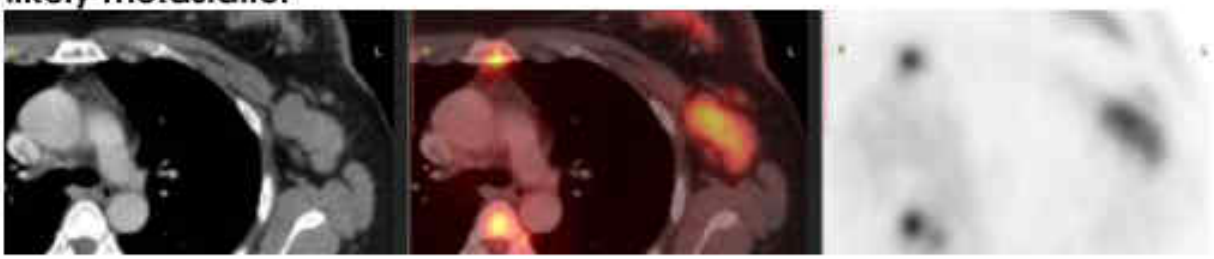
Regions of interest:

Multiple left breast FDG avid soft tissue nodules and masses, largest is seen at the outer upper and lower quadrants seen inseparable from the skin measuring about 8.7x6.1 cm with SUVmax~10.1, likely site of primary neoplasm for histopathological assessment.

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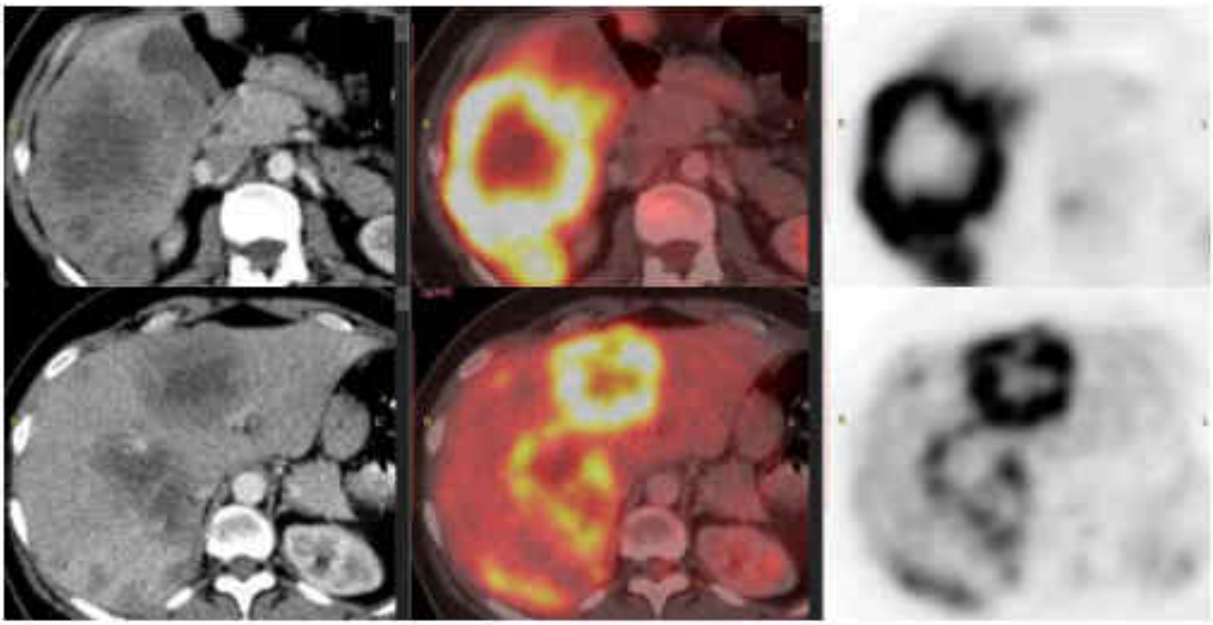


Multiple pathologically enlarged FDG avid left axillary and subpectoral LNs measuring about 4.9x2.5 cm with SUVmax~5.2, likely metastatic.

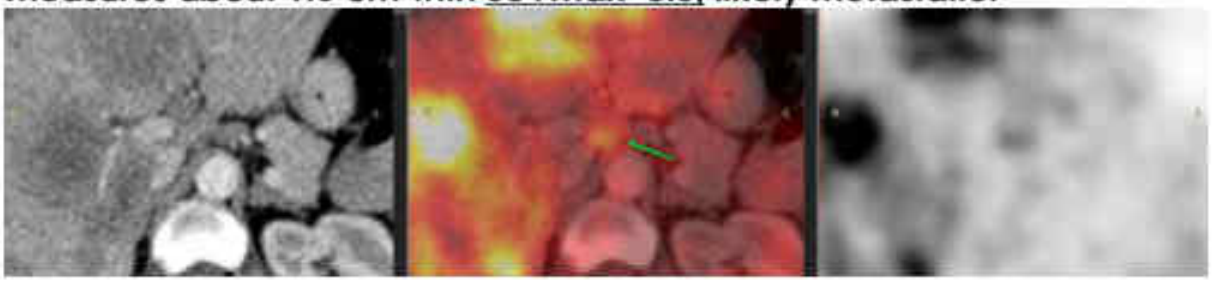


Multiple FDG avid metastatic hypodense hepatic focal lesions with mild peripheral enhancement, largest is seen at segment V and VI measuring about 8.1x7 cm with SUVmax~11.7.

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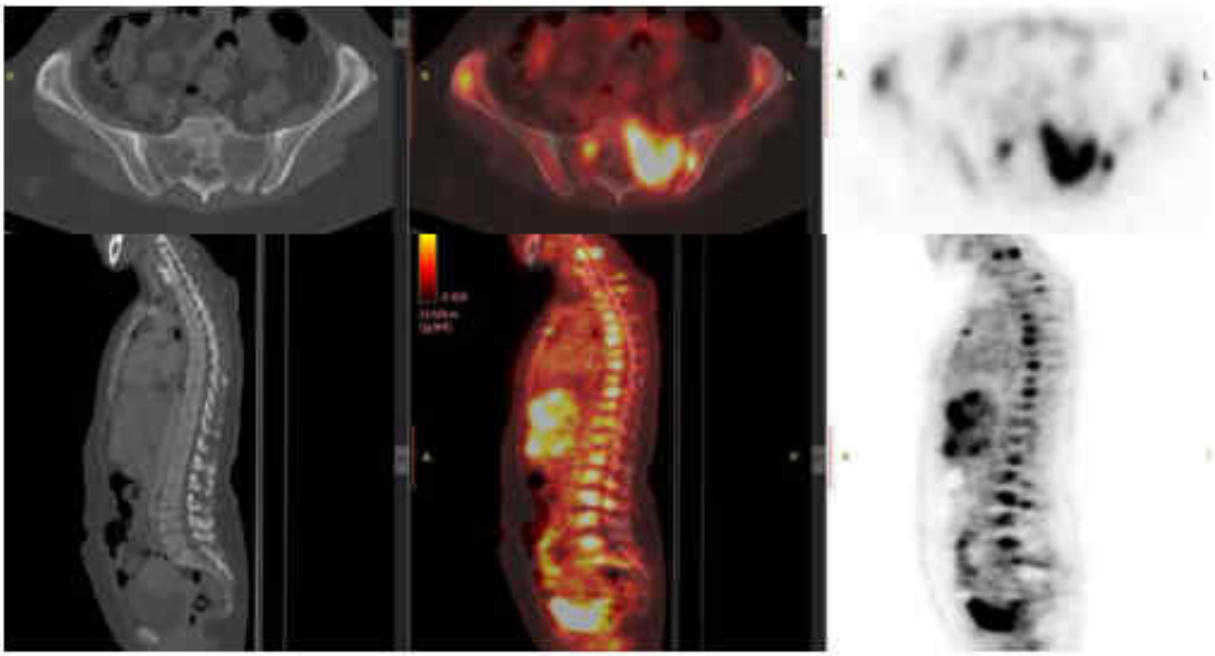


Few FDG avid abdominal porta-hepatis lymph nodes, largest measures about 1.8 cm with SUVmax~3.8, likely metastatic.



Multiple FDG avid lytic and bone marrow based osseous deposits are seen involving the spine, right humerus, right scapula, sternum, bilateral ribs, sacrum, multiple pelvic bones and both femora with SUVmax~11.9 measured at the sacrum.

Pat. Name: Walaa Mohamed Farag Sebaey	Pat. ID: 3607480
Procedure Name: PET - CT. Nuclear	Date: 23/07/2026



Head and Neck:

* No hypermetabolic cervical lymphadenopathy could be detected.
* Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
* The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

* No hypermetabolic mediastinal lymphadenopathy.
* No FDG avid lung nodules or masses.
* There are no active pleural abnormalities.
* Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

* Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
* The pancreas shows normal FDG avidity.
* There are no active adrenal nodules.
* Physiologic FDG excretion is seen in the kidneys and bladder.

Pat. Name: Walaa Mohamed Farag Sebaey	Pat. ID: 3607480
Procedure Name: PET - CT. Nuclear	Date: 23/07/2026

Conclusion:

- **FDG avid multicentric left breast neoplastic lesions, likely the site of primary, for histopathological verification.**
- **Multiple FDG avid metastatic nodal, osseous and hepatic lesions, as detailed above.**

Thank you for referral

Mahmoud A. Nouh
Nuclear medicine specialist
National Cancer Institute.

Ahmed Zaher , MD
Professor of nuclear medicine
National Cancer Institute

For any inquiries regarding the findings in the report please contact us on **radreport@cairoscan.com.eg**

مركز الجيزة للأشعة
GIZA SCAN

أ.د/ حســـــن الكيكى
أ.د/ محمد عبد اللطيف
د./ رانـــيـــا حـــمدى
د./ أيمـــن عبد الوهاب

ID : 11167143	NAME: ولاء عبد الحليم سماعيل
Date : 15/07/2026	Prof.Dr : Ashraf Ismail

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E-mail:giza.scan@yahoo.com

جيزا سكان
GIZA SCAN

أ.د/ حسن الكيكى
أ.د/ محمد عبد اللطيف
د./ رانيا حمدى
د./ أيمن عبد الوهاب

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E-mail:giza.scan@yahoo.com

Pat. Name: Walaa Mohamed Farag Sebaey	Pat. ID: 3607480
Procedure Name: PET - CT. Nuclear	Date: 23/07/2026

Whole Body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- Female patient with vertebral lesions on MRI.
- **PET/CT requested for assessment.**



Technique:

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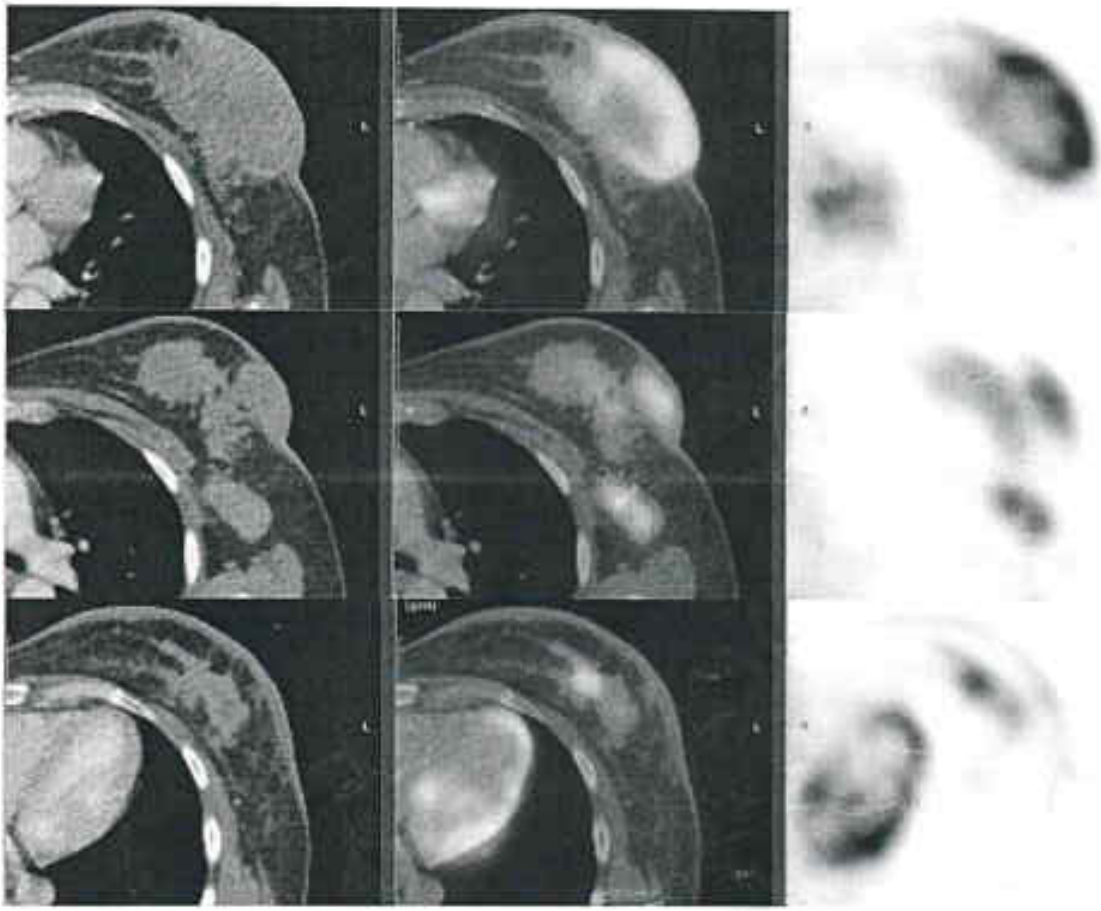
Report

Hepatic reference activity: 2.5

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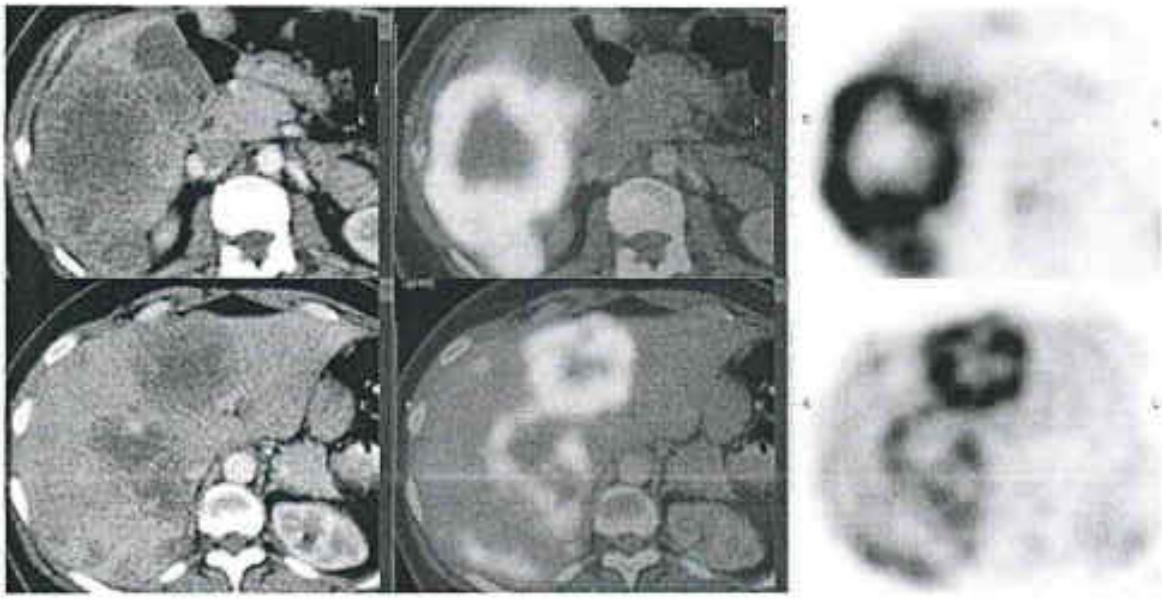


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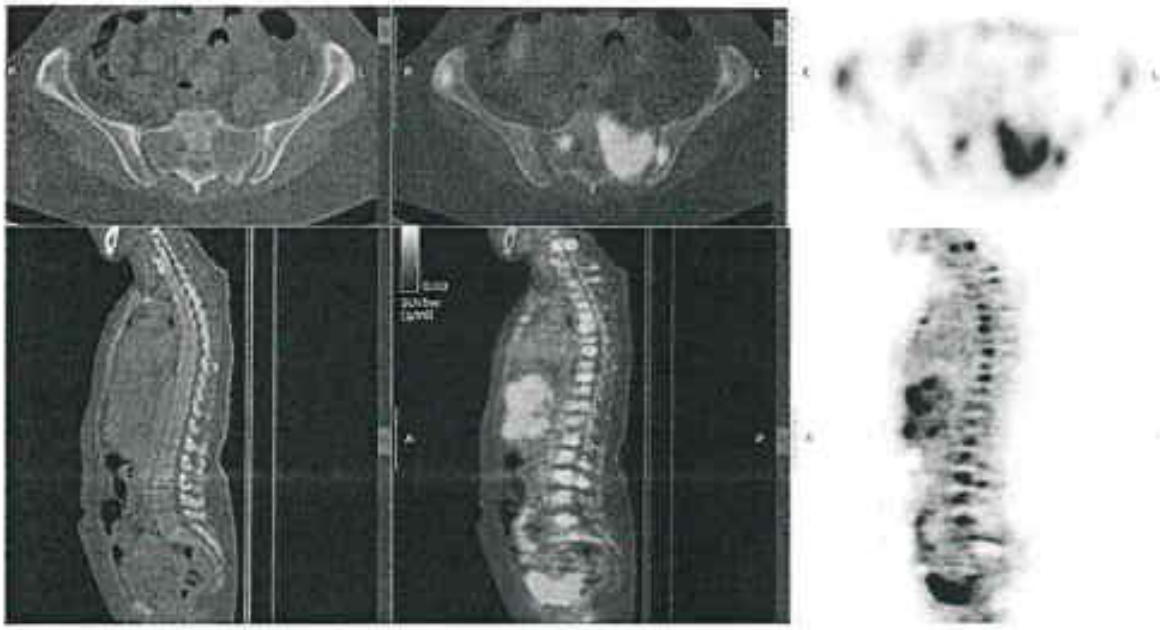


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- The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- No hypermetabolic mediastinal lymphadenopathy.
- No FDG avid lung nodules or masses.
- There are no active pleural abnormalities.
- Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- The pancreas shows normal FDG avidity.
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- Physiologic FDG excretion is seen in the kidneys and bladder.

Pat. Name: Walaa Mohamed Farag Sebaey	Pat. ID: 3607480
Procedure Name: PET - CT. Nuclear	Date: 23/07/2026

Conclusion:

- FDG avid multicentric left breast neoplastic lesions, likely the site of primary, for histopathological verification.
- Multiple FDG avid metastatic nodal, osseous and hepatic lesions, as detailed above.

Thank you for referral

Mahmoud A. Nouh
Nuclear medicine specialist
National Cancer Institute.

Ahmed Zaher , MD
Professor of nuclear medicine
National Cancer Institute

جمهورية مصر العربية
بطاقة تحقيق الشخصية



ولاء
محمد فرج سياعى فرج
٣ ش ماهر عبدالرحيم من فرج يوسف
دار السلام - القاهره

٢٧٨١٠٢٣٠١٠٣٩٤٥

LJ2221939



٢٧٨١٠٢٣٠١٠٣٩٤٥ ٢٠٢٥/٠٩
حاصله على بكالوريوس تجاره
شعبة محاسبه
انثى مسلمة متزوجة
وائل محمود محمد موسي
البطاقة سارية حتى ٢٠٣٢/٠٩/٠٨
١

جمهورية مصر العربية

بطاقة تحقيق الشخصية



ولاء

محمد فرج سباعى فرج

٧ ش صابر ابوشاهين

دار السلام - القاهره

١٩٧٨/١/٢٣

٢٧٨١٠٢٣٠١٠٣٩٤٥

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٢٧٨١٠٢٣٠١٠٣٩٤٥ ٢٠١٤/٠٩

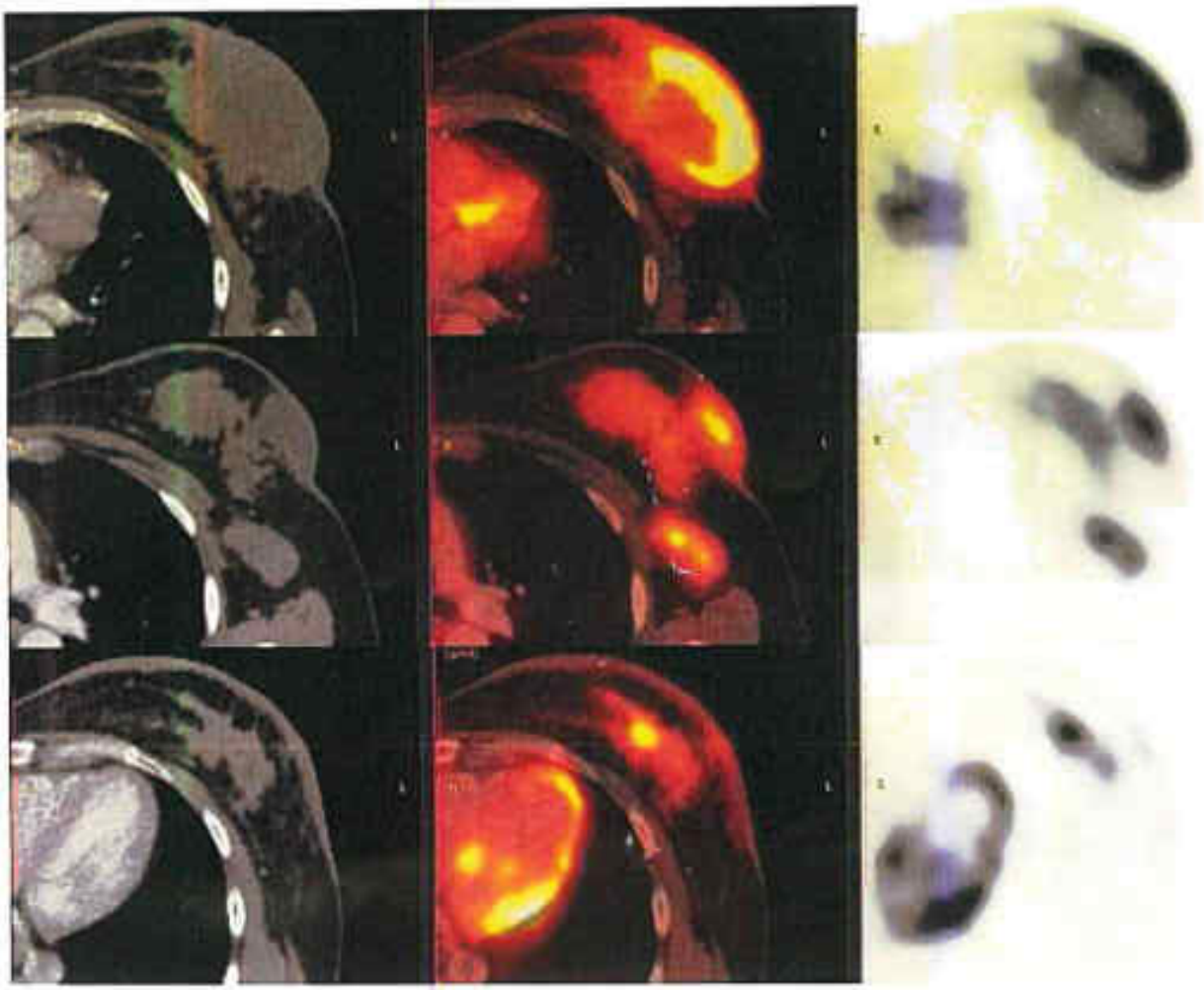
حاصله على بكالوريوس تجاره

شعبة محاسبه

انثى مسلمة متزوجة

وائل محمود محمد موسى

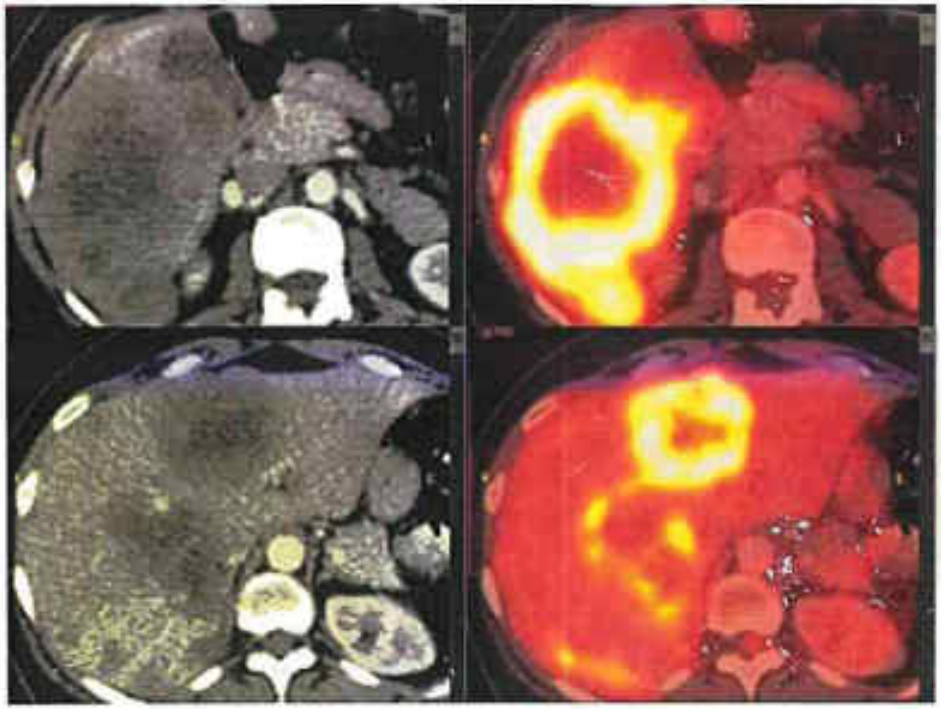
١ البطاقة سارية حتى ٢٠٢١/٠٩/٠٢



Multiple pathologically enlarged FDG avid left axillary and subpectoral LNs measuring about 4.9x2.5 cm with SUVmax~5.2, likely metastatic.



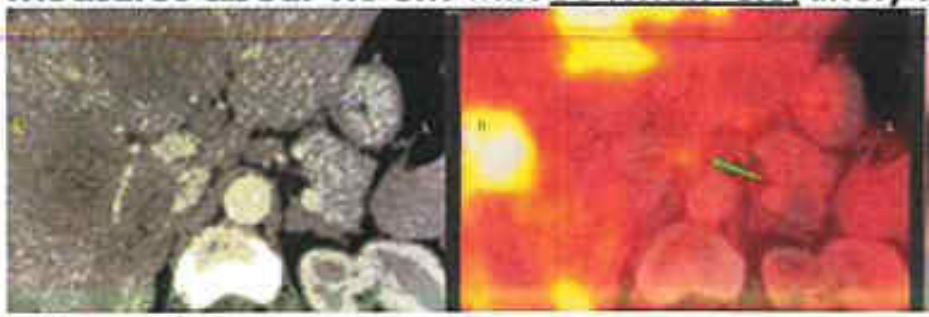
Multiple FDG avid metastatic hypodense hepatic focal lesions with mild peripheral enhancement, largest is seen at segment V and VI measuring about 8.1x7 cm with SUVmax~11.7.





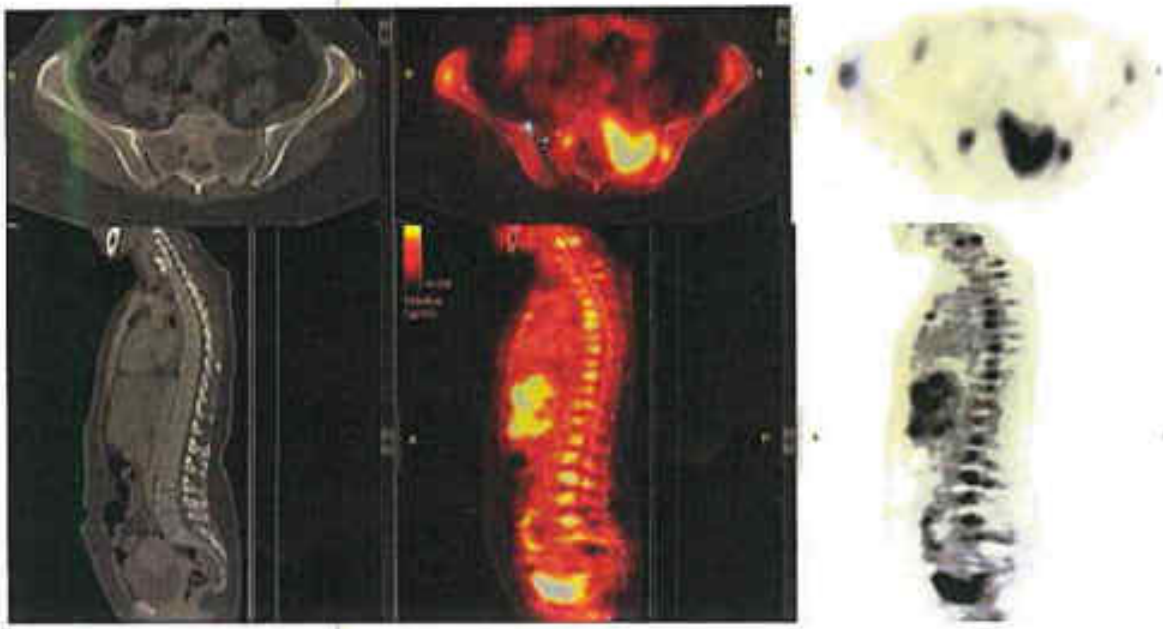


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Multiple FDG avid lytic and bone marrow based osseous deposits are seen involving the spine, right humerus, right scapula, sternum, bilateral ribs, sacrum, multiple pelvic bones and both femora with SUVmax~11.9 measured at the sacrum.



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- No hypermetabolic cervical lymphadenopathy could be detected.
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Whole Body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- Female patient with vertebral lesions on MRI.
- **PET/CT requested for assessment.**

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.



Report

Hepatic reference activity: 2.5

Regions of interest:

Multiple left breast FDG avid soft tissue nodules and masses, largest is seen at the outer upper and lower quadrants seen inseparable from the skin measuring about 8.7x6.1 cm with SUVmax~10.1, likely site of primary neoplasm for histopathological assessment.

I 1647

Conclusion:

- FDG avid multicentric left breast neoplastic lesions, likely the site of primary, for histopathological verification.
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مركز الجيزة للأشعة
GIZA SCAN

أ.د/ حســـــن الكيكى
أ.د/ محمد عبد اللطيف
د./ رانـــيـــا حـــمدى
د./ أيمـــن عبد الوهاب

ID : 11167143	NAME: ولاء محمد فرج سباعي
Date : 15/07/2026	Prof.Dr : Ashraf Ismail

MRI EXAMINATION OF THE LUMBO-SACRAL SPINE

Technique of examination:

- Sagittal T1 & T2 WIs
- Axial T2 WIs
- No IV contrast was given.

MRI Findings:

- There are multiple dorsal and lumbar vertebral body masses noted showing low T1 and low to intermediate T2 signal concerning for neoplastic lesions. No vertebral body collapse, no evident extra-vertebral component or vertebral canal extension. No neural compromise or foraminal narrowing.
- Also noted is a partially imaged left sacral mass showing the same signal characteristics. The outline of the mass can be seen in full on the scout images and approximately measures 5cm. Dedicated imaging is advised to evaluate for local infiltration and neural compromise if clinically found relevant. The scout images also show multiple concerning iliac bone small lesions.
- Preserved lumbar lordotic curvature.
- Normal diameters of the bony lumbar spinal canal.
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- Normal MRI appearance of the conus medullaris and cauda equina nerve roots.
- No paraspinal signal abnormalities.
- Uterine fibroids noted.

Opinion:

- ***Multiple bony lesions concerning for metastatic disease for further evaluation. No lumbar neural compromise.***
- ***Partially imaged large sacral mass – better evaluated by dedicated imaging.***

Kind regards,
Dr. Khaled Adel, Consultant radiologist

٥ ش مراد - ميدان الجيزة تليفاكس : ٣٧٧٤٦٠٢٠ - ٣٧٧٤٦٠٢١ - ٣٥٦٨٨٩٥٤ موبايل: ٠١١٤٠٧٧٢٢٦٨
E-mail:giza.scan@yahoo.com

GUSTAVE ROUSSY
Gustave Roussy International Egypt - Pathology Lab
جوستاف روسي الدولية للتكنولوجيا - مصر معمل البيثولوجي
جوستاف روسي

Patient Name	ولاء محمد فرج	**Case No.**	344973
Sex/Age	Female 47 Y	**Patient No.**	25351
Registered Date	30/7/2026 09:36:10 AM	**Referred By Dr.**	Interventional Radiology
Validation Date	17/8/2026 02:36:28 PM	**Branch**	Gustave Roussy
Reporting Date	18/8/2026 09:42:11 AM	**Pathology No.**	2396/26

Pathology Report

Clinical Data: Left breast multicentric mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 30/07/2026, at 1:40 pm. Cold ischemia time is 20 seconds.

Gross Examination:
- Fragmented grey, white tissue cores, ranging in size between 0.2 cm & 1 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
- **Histologic type:** Invasive duct carcinoma with mucinous component.
- **Histologic grade:** 2 (differentiation 2, pleomorphism 2, mitosis 2).
- **Ductal carcinoma in situ:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.

Results:
*** IPT (All controls showed appropriate reactivity):**
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in <1% of tumor cells (score 2/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

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1+).
- Ki67%: 40%.

Conclusion:
Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma with mucinous component, Grade 2.

IPT:
- ER: Positive (score 8/8).
- PR: Negative (score 2/8).
- Her2: Low (score 1+).
- Ki67%: 40%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
*** Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Patient Name	ولاء محمد فرج	**Case No.**	344973
Sex/Age	Female 47 Y	**Patient No.**	25351
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Signature
Prof. Dr. Maha Guimei.

Patient's Name:	**Walaa Mohammed Farag Sabai Farag**
Date:	**2026-07-28**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left palpable mass

Findings:

Right breast.

- Heterogenously dense breast parenchyma (ACR c)
- UOQ oval obscured margins lesions
- UOQ grouped macrocalcifications
- **Ultrasound** demonstrated a circumscribed oval hypoechoic lesion at 10 oclock zone B measuring 17 x 10 mm
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Heterogenously dense breast parenchyma (ACR c)
- large infiltritave dense irregular speculated mass with pathological pleomorphic calcifications involving most of the UOQ and upper central region
- thickened skin , coarsened trabeculations
- **Ultrasound** demonstrated central and outer breast 10- 5 o'clock large irregular speculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study measuring about 7.8 x 4.3 cm reaching the nipple.
- Thickened skin reaching 10 mm with coarsened trabecular and minimal subdermal edema.
- Multiple innumerable pathological axillary LNs are noted at level 1, 2 & 3, the largest at level 1 measuring 2.3 x 17 mm

Conclusion:-

- ***Right breast (BIRADS 3): 10 oclock probably benign lesion, likely fibroadenoma***
- ***Left breast (BIRADS 5): highly suspicious lesion with ipsilateral pathological level 1, 2 , 3 axillary nodes for histopathological correlation***

dr farah elshobaky

Modern Lab
Clinical Laboratory

المعمل الحديث
للتحاليل الطبية
د/ ماهر صدقى
اخصائى تحاليل طبية

Patient ID.	1026072010180	Patient Name	السيدة/ ولاء محمد فرج سباعي
Request Date	20-Jul-2026	Age / Sex	47 Years / Female
Printed In	20-Jul-2026	Referred By	

REPORT

Blood Glucose

			Reference Range :
F.Bl.G. (Fasting Blood Glucose)	:	108 mg/dL	Normal: 70 - 110 mg/dL
Urine Sample			
Glucose	:	No Sample	Nil
Acetone	:	No Sample	Nil

KIDNEY FUNCTIONS

Serum Creatinine	:	0.92 mg/dL	Normal: 0.6 - 1.1 mg/dL

Doctor's signature:
Maher Sedky
Thanks

العنوان : ١ ش السكة الحديد بجوار محطة مترو الزهراء – م.القديمة
ترخيص رقم : ١٤٢٦٢ ت : ٢٧٢٢٨٢٤٠ – موبايل : ٠١٢٢٧٥١٤٠٦٨

Modern Lab
Clinical Laboratory

المعمل الحديث
للتحاليل الطبية
د/ ماهر صدقى
اخصائى تحاليل طبية

Patient ID.	1026071710074	Patient Name	مدام/ ولاء محمد فرج سباعي
Request Date	17-Jul-2026	Age / Sex	47 Years / Female
Printed In	18-Jul-2026	Referred By	أ.د/ اشرف اسماعيل

REPORT

LIVER FUNCTIONS

				Reference Range :
SGOT (Aspartate Aminotransferase)	:	60 U/L	High	Normal : Up to 31 U/L
SGPT (Alanine Aminotransferase)	:	47 U/L	High	Normal : Up to 31 U/L

KIDNEY FUNCTIONS

Serum Creatinine	:	0.88 mg/dL		Normal: 0.6 - 1.1 mg/dL
Blood Urea	:	32 mg/dL		Normal : 15 - 45 mg/dL

SERODIAGNOSIS

CRP (C-Reactive Protein)	:	Positive 37		Normal : Less than 5.0 mg/dL

Erythrocyte Sedimentation Rate

First Hour	:	60 mms	High	Normal: Up to 7 mms
Second Hour	:	122 mms	High	Normal: Up to 15 mms

Doctor's signature:
Maher Sedky
Thanks

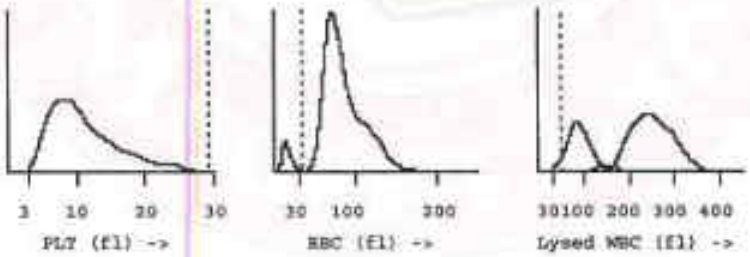
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ترخيص رقم : ١٤٢٦٢ ت : ٢٧٢٢٨٢٤٠ – موبايل : ٠١٢٢٧٥١٤٠٦٨

Modern Lab
Clinical Laboratory

المعمل الحديث
للتحاليل الطبية
د/ ماهر صدقى
اخصائى تحاليل طبية

ID = WALAA MOHAMED
ID2 = D/ASHARF
SEQ = 6454 FEMALE OT
DATE= 18/07/2026
TIME= 12:00:00

			Normal ranges
RBC =	4.60	10^{12}/l	3.80 : 5.70
HGB =	12.5	g/dl	12.0 : 16.0
HCT =	38.5	%	36.0 : 45.0
MCV =	83.6	fl	81.0 : 101.0
MCH =	27.3	pg	27.0 : 32.0
MCHC=	32.6	g/dl	31.5 : 34.5
RDWa=	66.5	fl	30.0 : 150.0
RDW%=	12.0	%	11.0 : 16.0
PLT =	344	10^{9}/l	150 : 450
MPV =	11.0	fl	8.0 : 11.0
PDW =	14.1	fl	0.1 : 99.9
PCT =	0.38	%	0.01 : 9.99
LPCR=	35.2	%	0.1 : 99.9
WBC =	6.4	10^{9}/l	4.0 : 11.0
LYM%=	27.7	%	20.0 : 45.0
MID%=	6.6	%	2.0 : 18.0
GRA%=	65.7	%	40.0 : 86.0
LYM =	1.7	10^{9}/l	1.0 : 4.0
MID =	0.5	10^{9}/l	0.2 : 1.6
GRAN=	4.2	10^{9}/l	2.0 : 7.0



العنوان : ١ ش السكة الحديد بجوار محطة مترو الزهراء – م.القديمة
ترخيص رقم : ١٤٢٦٢ ت : ٢٧٢٢٨٢٤٠ – موبايل : ٠١٢٢٧٥١٤٠٦٨

جيزا سكان
GIZA SCAN

أ.د/ حسن الكيكى
أ.د/ محمد عبد اللطيف
د./ رانيا حمدى
د./ أيمن عبد الوهاب

ID : 11167143	NAME:
Date : 15/07/2026	Prof.Dr : Ashraf Ismail

MRI EXAMINATION OF THE LUMBO-SACRAL SPINE

Technique of examination:

- Sagittal T1 & T2 WIs
- Axial T2 WIs
- No IV contrast was given.

MRI Findings:

- There are multiple dorsal and lumbar vertebral body masses noted showing low T1 and low to intermediate T2 signal concerning for neoplastic lesions. No vertebral body collapse, no evident extra-vertebral component or vertebral canal extension. No neural compromise or foraminal narrowing.
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٥ ش مراد - ميدان الجيزة تليفاكس : ٣٧٧٤٦٠٢٠ - ٣٧٧٤٦٠٣١ ٢٥٦٨٨٩٥١ موبايل: ٠١١٠٧٧٢٢٦٨
E-mail:giza.scan@yahoo.com

Pat. Name: Walaa Mohamed Farag Sebaey	Pat. ID: 3607480
Procedure Name: PET - CT. Nuclear	Date: 23/07/2026

Whole Body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- Female patient with vertebral lesions on MRI.
- **PET/CT requested for assessment.**



Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.

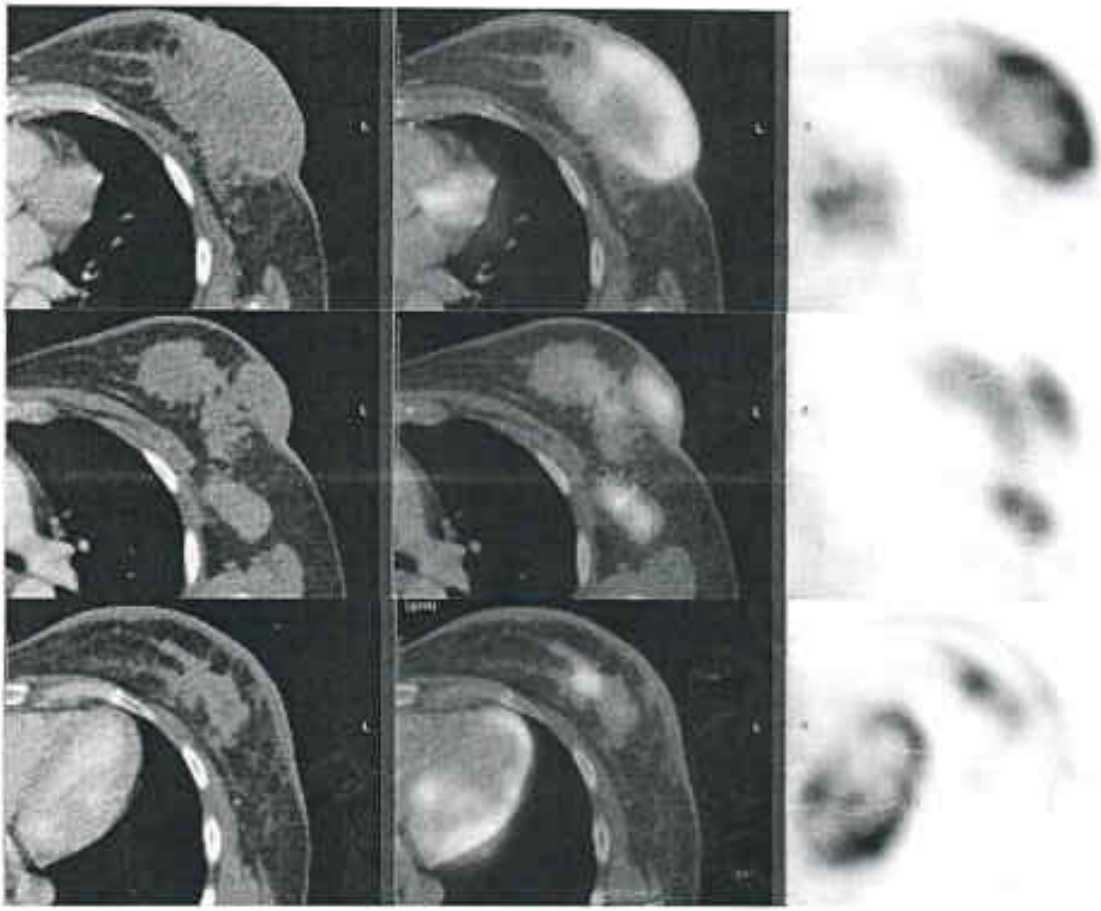
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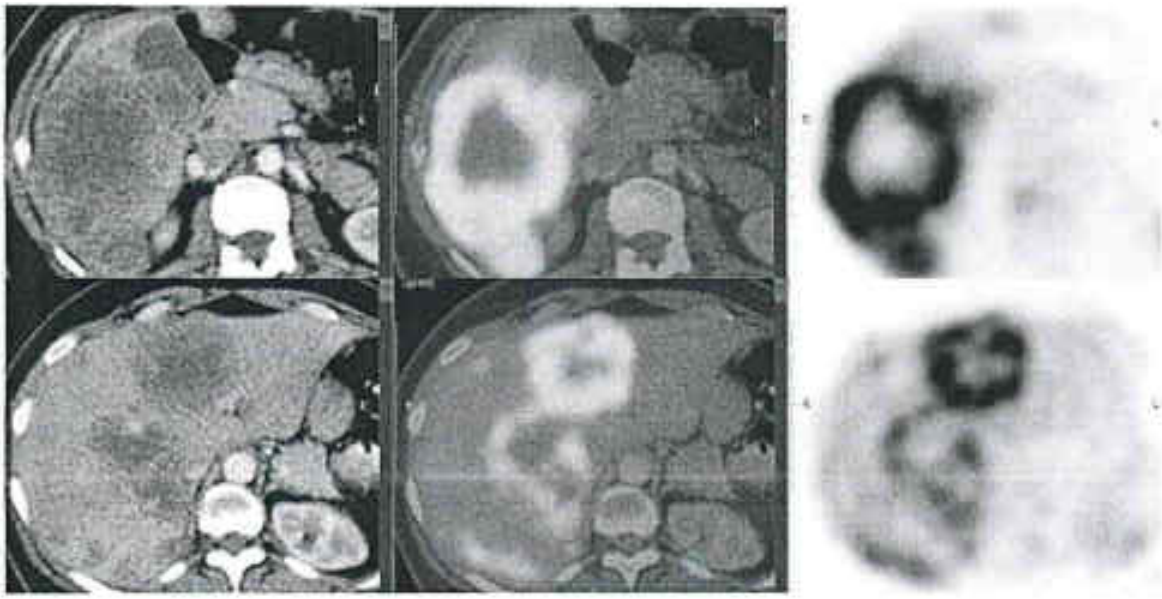


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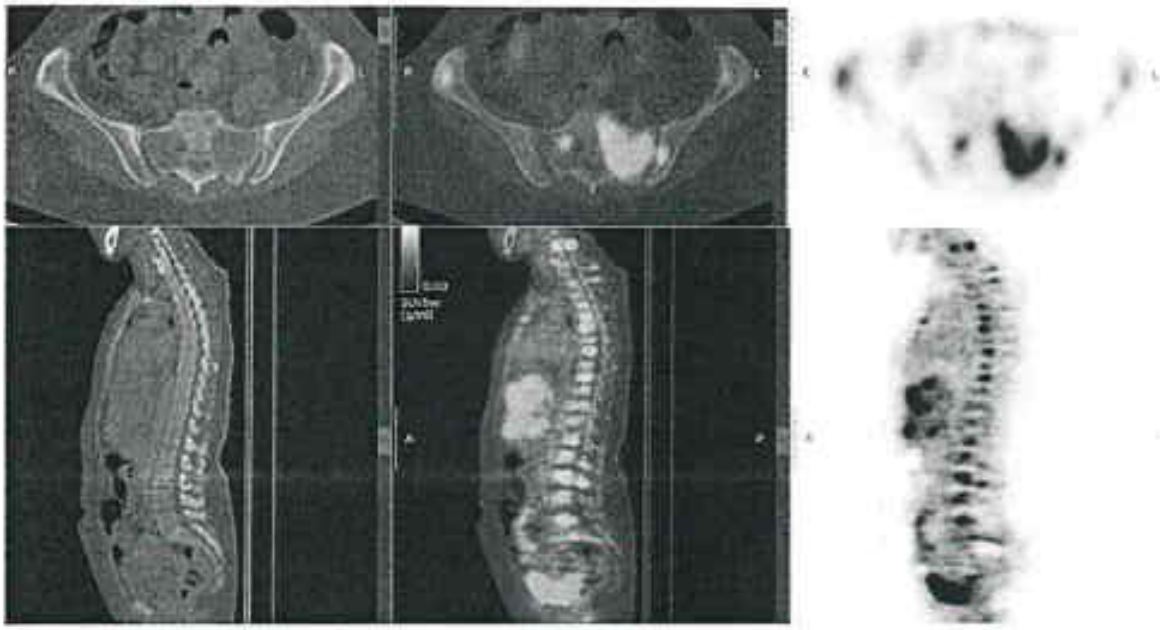


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جمهورية مصر العربية
بطاقة تحقيق الشخصية



ولاء
محمد فرج سياعى فرج
٣ ش ماهر عبدالرحيم من فرج يوسف
دار السلام - القاهره

٢٧٨١٠٢٣٠١٠٣٩٤٥

LJ2221939



٢٧٨١٠٢٣٠١٠٣٩٤٥ ٢٠٢٥/٠٩
حاصله على بكالوريوس تجاره
شعبة محاسبه
انثى مسلمة متزوجة
وائل محمود محمد موسي
البطاقة سارية حتى ٢٠٣٢/٠٩/٠٨
١

جيزا سكان
GIZA SCAN

أ.د/ حسن الكيكى
أ.د/ محمد عبد اللطيف
د./ رانيا حمدى
د./ أيمن عبد الوهاب

ID : 11167143	NAME:
Date : 15/07/2026	Prof.Dr : Ashraf Ismail

MRI EXAMINATION OF THE LUMBO-SACRAL SPINE

Technique of examination:

- Sagittal T1 & T2 WIs
- Axial T2 WIs
- No IV contrast was given.

MRI Findings:

- There are multiple dorsal and lumbar vertebral body masses noted showing low T1 and low to intermediate T2 signal concerning for neoplastic lesions. No vertebral body collapse, no evident extra-vertebral component or vertebral canal extension. No neural compromise or foraminal narrowing.
- Also noted is a partially imaged left sacral mass showing the same signal characteristics. The outline of the mass can be seen in full on the scout images and approximately measures 5cm. Dedicated imaging is advised to evaluate for local infiltration and neural compromise if clinically found relevant. The scout images also show multiple concerning iliac bone small lesions.
- Preserved lumbar lordotic curvature.
- Normal diameters of the bony lumbar spinal canal.
- Preserved height and T2 signal brightness of the lumbar intervertebral discs.
- No evidence of disc herniations or significant bulges.
- Normal MRI appearance of the conus medullaris and cauda equina nerve roots.
- No paraspinal signal abnormalities.
- Uterine fibroids noted.

Opinion:

- ***Multiple bony lesions concerning for metastatic disease for further evaluation. No lumbar neural compromise.***
- ***Partially imaged large sacral mass – better evaluated by dedicated imaging.***

Kind regards,
Dr. Khaled Adel, Consultant radiologist

٥ ش مراد - ميدان الجيزة تليفاكس : ٣٧٧٤٦٠٢٠ - ٣٧٧٤٦٠٣١ ٢٥٦٨٨٩٥١ موبايل: ٠١١٠٧٧٢٢٦٨
E-mail:giza.scan@yahoo.com

Pat. Name: Walaa Mohamed Farag Sebaey	Pat. ID: 3607480
Procedure Name: PET - CT. Nuclear	Date: 23/07/2026

Whole Body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- Female patient with vertebral lesions on MRI.
- **PET/CT requested for assessment.**



Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.

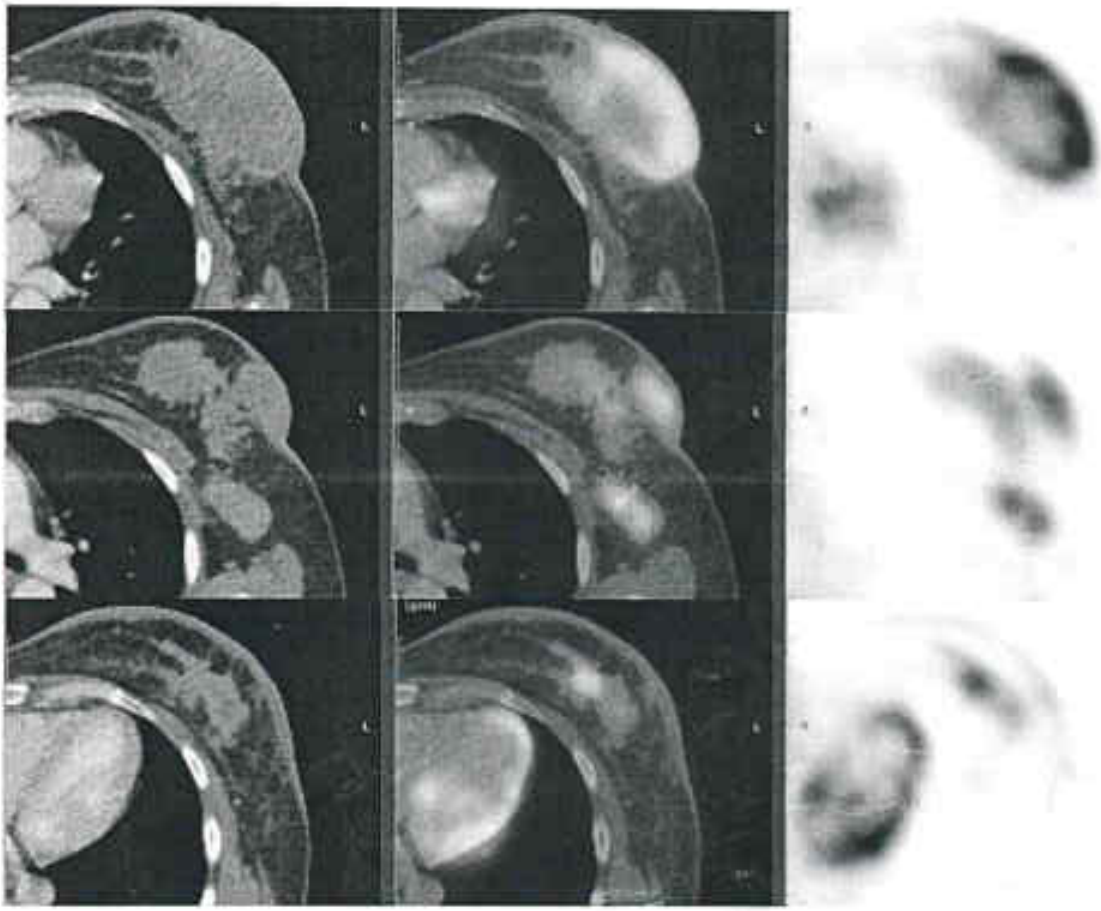
Report

Hepatic reference activity: 2.5

Regions of interest:

Multiple left breast FDG avid soft tissue nodules and masses, largest is seen at the outer upper and lower quadrants seen inseparable from the skin measuring about 8.7x6.1 cm with SUVmax~10.1, likely site of primary neoplasm for histopathological assessment.

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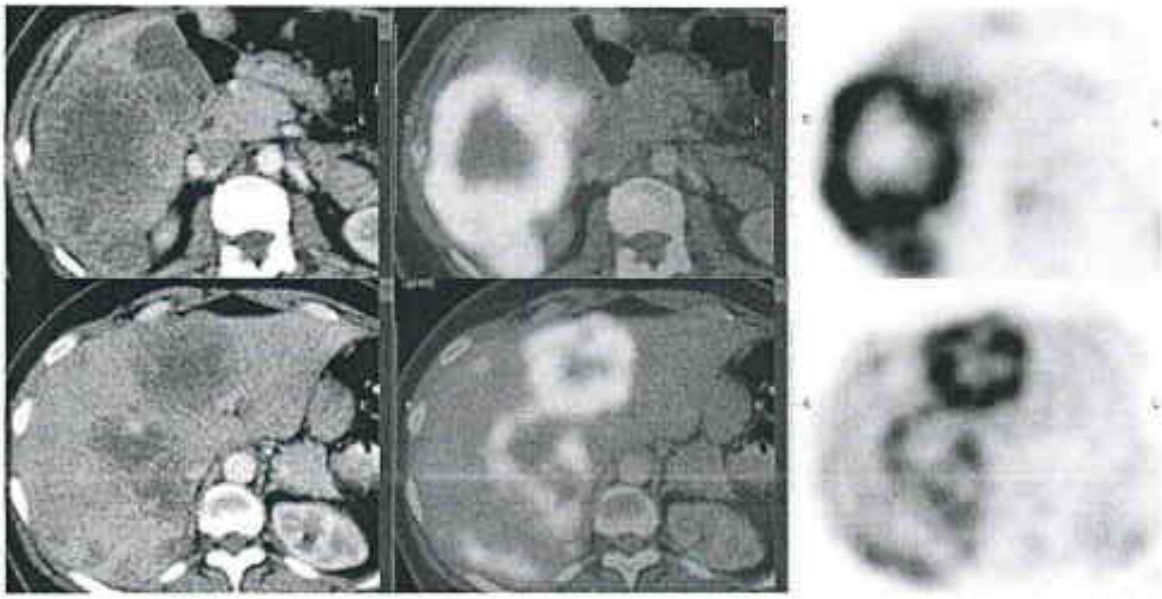


Multiple pathologically enlarged FDG avid left axillary and subpectoral LNs measuring aboui 4.9x2.5 cm with SUVmax~5.2, likely metastatic.



Multiple FDG avid metastatic hypodense hepatic focal lesions with mild peripheral enhancement, largest is seen at segment V and VI measuring about 8.1x7 cm with SUVmax~11.7.

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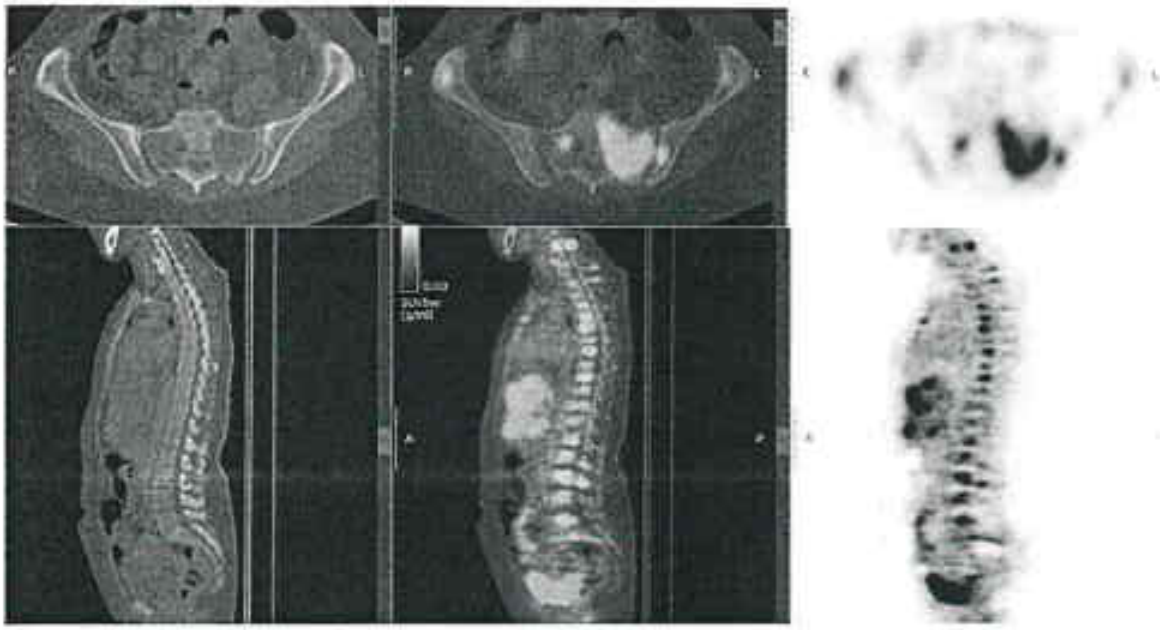


Few FDG avid abdominal porta-hepatis lymph nodes, largest measures about 1.8 cm with SUVmax~3.8, likely metastatic.



Multiple FDG avid lytic and bone marrow based osseous deposits are seen involving the spine, right humerus, right scapula, sternum, bilateral ribs, sacrum, multiple pelvic bones and both femora with SUVmax~11.9 measured at the sacrum.

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Head and Neck:

- No hypermetabolic cervical lymphadenopathy could be detected.
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.

Chest:

- No hypermetabolic mediastinal lymphadenopathy.
- No FDG avid lung nodules or masses.
- There are no active pleural abnormalities.
- Physiologic FDG uptake is noted in the heart.

Abdomen and Pelvis:

- Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- The pancreas shows normal FDG avidity.
- There are no active adrenal nodules.
- Physiologic FDG excretion is seen in the kidneys and bladder.

Pat. Name: Walaa Mohamed Farag Sebaey	Pat. ID: 3607480
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Conclusion:

- FDG avid multicentric left breast neoplastic lesions, likely the site of primary, for histopathological verification.
- Multiple FDG avid metastatic nodal, osseous and hepatic lesions, as detailed above.

Thank you for referral

Mahmoud A. Nouh
Nuclear medicine specialist
National Cancer Institute.

Ahmed Zaher , MD
Professor of nuclear medicine
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جمهورية مصر العربية
بطاقة تحقيق الشخصية



ولاء
محمد فرج سياعى فرج
٣ ش ماهر عبدالرحيم من فرج يوسف
دار السلام - القاهره

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حاصله على بكالوريوس تجاره
شعبة محاسبه
انثى مسلمة متزوجة
وائل محمود محمد موسي
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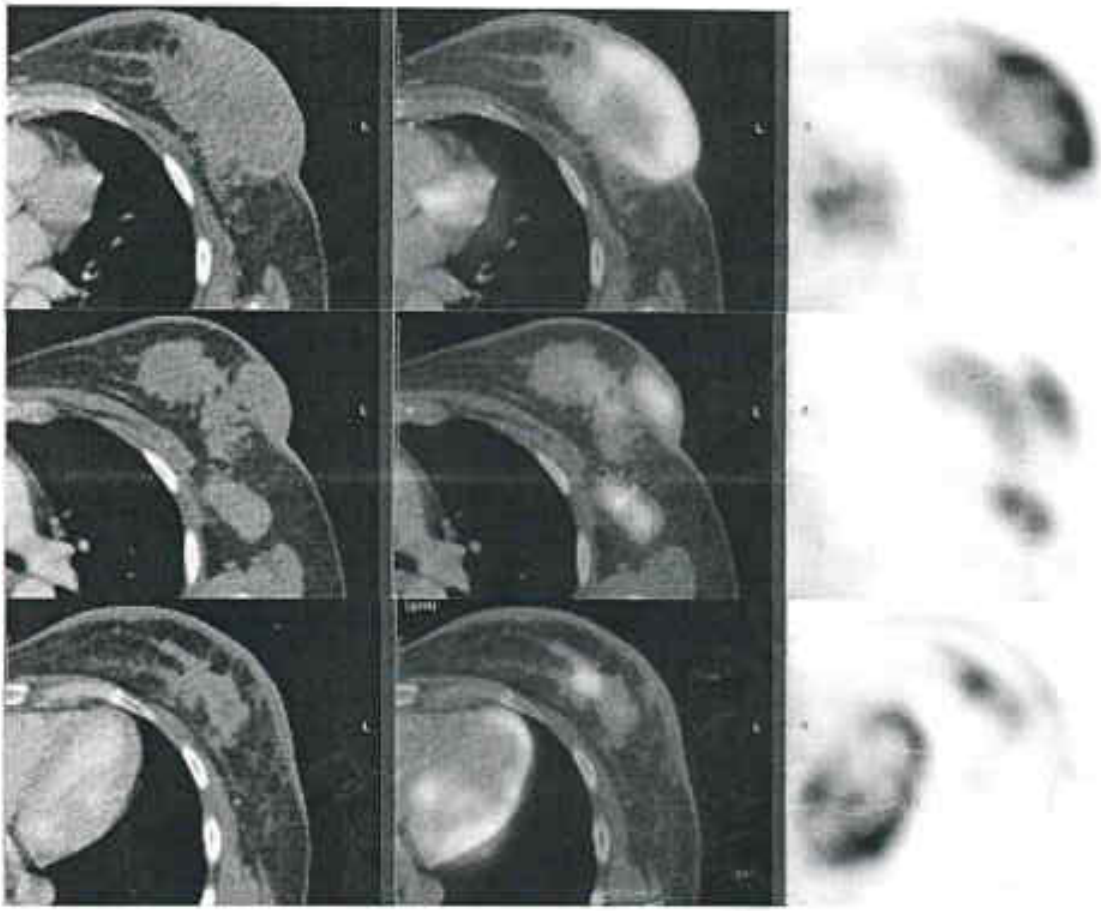
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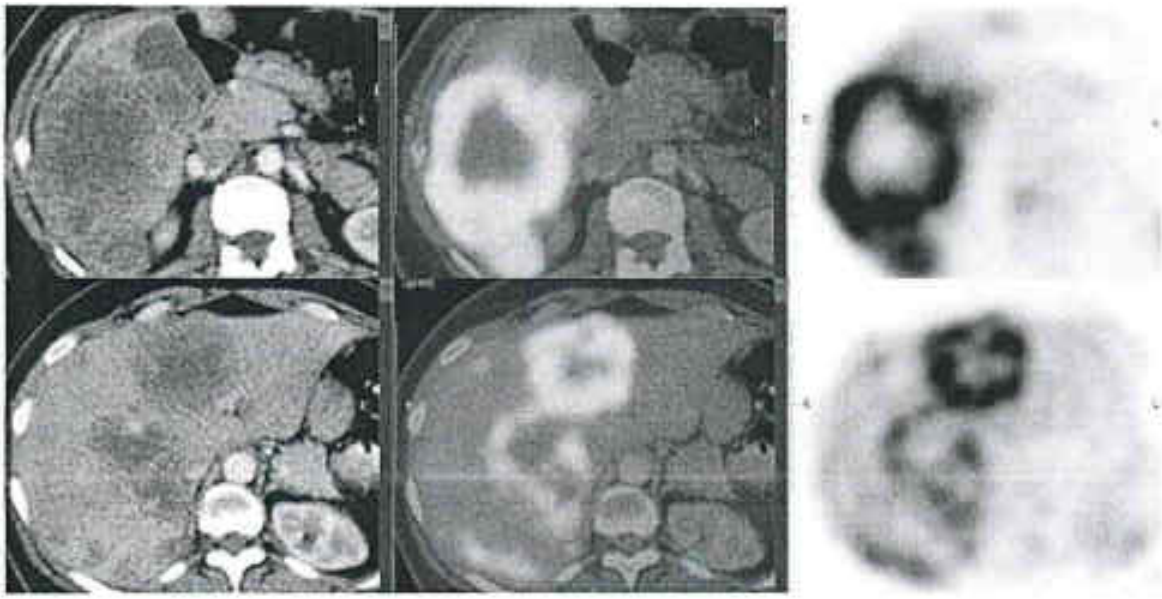


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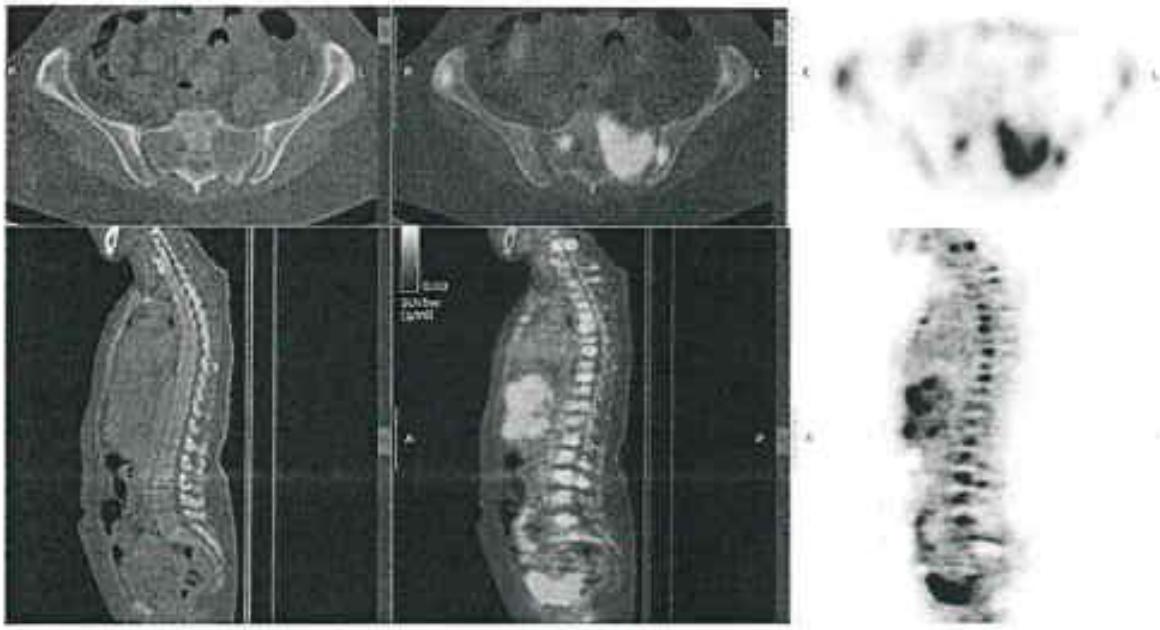


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